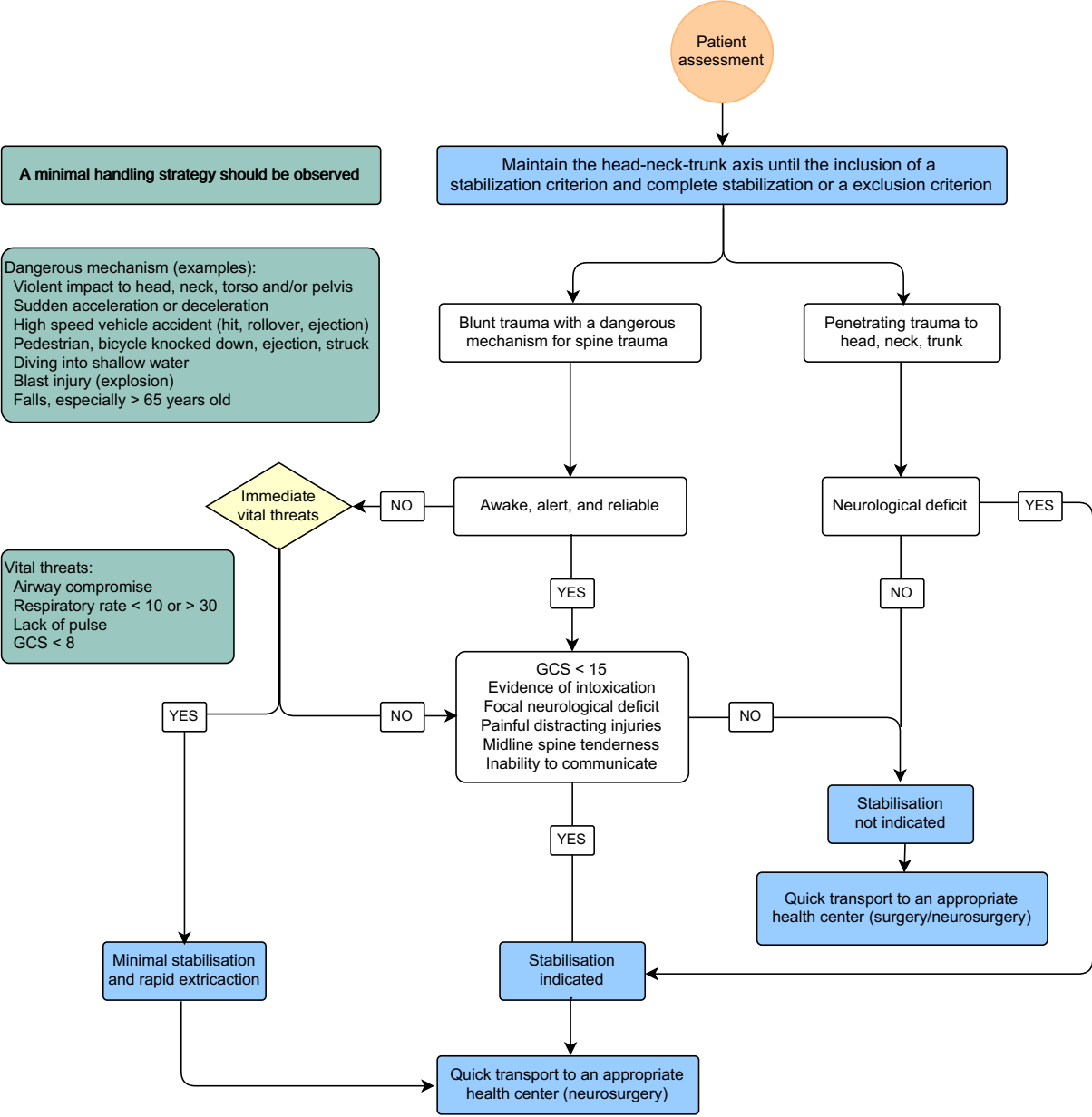


SPINE STABILISATION CRITERIA



Painful distracting injuries: Condition that produces enough pain to distract the patient from a secondary injury, like long bone fracture, visceral injury, large laceration, de-gloving or crush injury, large burns, severe pain, etc.

The means of immobilisation and stabilization can be the manual maintenance of the head, the padding or the head-immobiliser associated with another means of immobilisation, the rigid cervical collar associated with another means of immobilisation/extraction such as the vacuum mattress, the spineboard or the KED.

Other stabilisation means can be used for rapid extrication before immobilisation.

The scoop with straps is recommended for lifting on the scene and for transfer from the ambulance stretcher to the hospital stretcher (foster the scoop over the log-roll).

The use of a vacuum mattress as a means of immobilisation should be preferred. The spineboard should be reserved for very specific situations and as a means of extrication (source of discomfort for the patient with risk of involuntary spine movement and pressure sores-ulcers).

This protocol is based on the NEXUS criteria, the Canadian C-Spine Rule, the Wilderness Medical Society and of Kornhall D.K., Jørgensen, J.J., Brommeland T., Hyldmo, P.K., Asbjørnsen, H., Dolven, T., Hansen, T. & Jeppesen, E. (2017). The Norwegian guidelines for the prehospital management of adult trauma patients with potential spinal injury. Scandinavian Journal of Trauma, Resuscitation and Emergency Medicine, 25(2). <https://10.1186/s13049-016-0345-x>